# SPIRITUAL TRAUMA

*Facilitator Guide • 2026 evidence-current production edition*

## Purpose

This guide makes the creator's curriculum transferable without turning another facilitator into Stephanie. The presentation remains a psychoeducation group, not a diagnostic assessment, theological debate, or group trauma-processing protocol.

Core guidance must transfer: evidence status, definitions, slide purpose, clinical distinctions, cautions, transitions, containment, disclosure response, and referral or safety boundaries. Stephanie confetti is optional: humor, anecdotes, cartoons, cultural examples, memorable analogies, and creator-specific transitions.

### Learning objectives

- Differentiate spiritual struggle, adverse experience, spiritual or religious abuse, trauma-related responses, and PTSD without requiring participants to adopt a label.
- Recognize how power, coercion, betrayal, and loss of autonomy can affect trust, meaning, identity, relationships, and recovery engagement.
- Identify options that restore agency in beliefs, treatment, relationships, and healing, including retaining faith, redefining faith, remaining uncertain, or leaving religion or spirituality.
- Describe treatment and support options using evidence-calibrated categories.

### One-session and two-session use

| Format | Suggested scope | Required transition |
|---|---|---|
| One session | Slides 1–33 with discussion paced to available time. Shorten examples before removing agency/healing content. | Protect time for the "Great… Now What?" transition and closure. |
| Part 1 | Slides 1–18: systems and group processes, abuse/coercion, red flags, trauma language, distinctions. | At slide 18, summarize without asking for disclosure. Offer support and responsive containment. |
| Part 2 | Return at slide 18, then slides 19–33: SHAS vocabulary, effects, meaning, healing, treatments, trauma-informed care. | Reorient choice and confidentiality limits; do not ask participants to recap personal disclosures. |

### Group safety and containment

- Participants may pass, step out, or speak privately with staff. Do not require belief disclosure, testimony, or defense of a faith or nonfaith position.
- No participant serves as spokesperson for a religion, denomination, recovery fellowship, survivor group, or identity.
- Redirect graphic detail and individualized trauma processing. Acknowledge the person and arrange private follow-up.
- When current abuse or danger emerges, assess privately, follow facility and mandated-reporting policy, consult as required, and document appropriately.
- Do not adjudicate theology. Shift from "Is this belief true?" to "How did this function, and what was its impact?"
- Make grounding and containment available. Use them responsively rather than mandating exercises at predetermined points.

#### Suggested opening language

"This is a psychoeducation group about religious and spiritual struggle, harm, and recovery. You may participate generally, pass, or step out. You do not have to name or defend your beliefs, disclose personal experiences, or decide whether the word trauma fits you. Please avoid graphic details and advice to other participants. If something raises a concern about current safety, we can talk privately and follow program policy."

# Slide-by-slide facilitator guidance

## Slide 1: Spiritual Trauma

**Participant-facing content:** Spiritual Trauma | Trigger Warning: This topic may bring up strong reactions. You may participate, pass, or step out. You do not have to share personal experiences. Please talk with me, your therapist, or another staff member if you need support after group.

**Core guidance:** State the limits of confidentiality and program rules. Participants may pass, step out, or speak with staff. This is psychoeducation, not detailed trauma processing. Do not require disclosure or a belief statement.

**Stephanie confetti:** Keep the warm, conversational warning. Humor can lower tension only after safety and choice are clear.

**Source anchors:** SAMHSA (2014), Concept of Trauma and Guidance for a Trauma-Informed Approach.

## Slide 2: Visual / transition slide

**Core guidance:** Use guided discovery. Ask what participants associate with spiritual trauma, oppressive systems, or high-control groups. Distinguish cult from occult. Expand to ordinary group processes: conformity, authority, in-group/out-group dynamics, exploitation of vulnerability, and divisions within recovery communities. Avoid diagnosing a group from one feature.

**Stephanie confetti:** The Naked Pastor/book imagery and familiar examples can open discussion. Preserve the point that intelligent, psychologically healthy people can become involved in controlling groups when person, needs, timing, circumstances, and the group's promises intersect.

**Source anchors:** Ellis et al. (2022); SAMHSA (2014).

## Slide 3: Religion and Spirituality

**Participant-facing content:** Religion and Spirituality | Religion often includes shared beliefs, practices, traditions, and community. Spirituality often involves meaning, connection, values, the sacred, or the transcendent. They may overlap. A person may identify with either, both, or neither.

**Core guidance:** Treat religion and spirituality as overlapping, self-defined terms. Do not imply that either definition is universal across cultures.

**Stephanie confetti:** Invite participants to supply their own definitions without requiring disclosure.

**Source anchors:** Pargament (1997); SAMHSA (2014).

## Slide 4: Spirituality Can Be Part of Recovery

**Participant-facing content:** Spirituality Can Be Part of Recovery | For many people, spirituality or religion can support healing, connection, and purpose. Recovery can also be secular. Higher-Power or religious language may create barriers when it resembles earlier harm.

**Core guidance:** Spirituality may support recovery for many people, but it is not required. Name secular recovery pathways and acknowledge that Higher-Power language may carry painful associations.

**Stephanie confetti:** This is a natural place for "You can have Jesus and treatment/medication/the 12 Steps/questions, too."

**Source anchors:** SAMHSA (2019); Kelly et al. (2020).

## Slide 5: Spiritual Struggle, Abuse, and Trauma

**Participant-facing content:** Spiritual Struggle, Abuse, and Trauma | These experiences can overlap, but they are not interchangeable. Spiritual struggle can involve conflict about God or the divine, religious people or institutions, morality, doubt, frightening spiritual interpretations, meaning, or purpose. Struggle can be painful without involving abuse or trauma.

**Core guidance:** Introduce the middle category: struggle. The six RSS domains are divine, demonic, interpersonal, moral, doubt, and ultimate meaning. Use examples, not a questionnaire. Normalize doubt and questioning.

**Stephanie confetti:** A simple line works: "A struggle can hurt without somebody abusing you, and pain can matter without becoming a diagnosis."

**Source anchors:** Exline et al. (2014); Exline et al. (2022), RSS-14.

## Slide 6: A Quick Recap of Trauma

**Participant-facing content:** A Quick Recap of Trauma | Trauma can follow an event, series of events, or circumstances experienced as harmful or threatening, with lasting effects on well-being and functioning. “Big-T” and “little-t” are informal shorthand, not diagnostic categories. They can remind us that acute, chronic, cumulative, relational, coercive, and identity-disruptive experiences may matter. PTSD has specific diagnostic criteria.

**Core guidance:** Big-T/little-t is informal shorthand. Use it to challenge self-invalidation, then move to precise descriptions such as acute, chronic, cumulative, relational, coercive, betrayal-based, or identity-disruptive. PTSD requires specific exposure and symptom criteria.

**Stephanie confetti:** Preserve the creator’s validating purpose rather than turning this into a diagnostic lecture.

**Source anchors:** Lumley et al. (2022); SAMHSA (2014); APA DSM-5-TR (2022).

## Slide 7: A Quick Recap of Trauma

**Participant-facing content:** A Quick Recap of Trauma | Trauma can affect individuals, families, communities, and identity groups. It can affect stress responses, emotions, thoughts, relationships, identity, and physical well-being. Responses vary, and recovery is possible. Abuse is never your fault.

**Core guidance:** Use may/can language. Trauma responses vary and are treatable. Avoid biological determinism. Reinforce that abuse is never the participant’s fault.

**Stephanie confetti:** The creator may paraphrase “the body keeps the score” as cultural shorthand, but do not present the book title as a universal clinical rule.

**Source anchors:** SAMHSA TIP 57 (2014); NIDA (2024).

## Slide 8: Spiritual Abuse, Adverse Experiences, and Trauma

**Participant-facing content:** Spiritual Abuse, Adverse Experiences, and Trauma | Spiritual or religious abuse: using beliefs, practices, relationships, community structures, or spiritual authority to coerce, control, exploit, intimidate, silence, punish dissent, or facilitate harm. Adverse religious or spiritual experience: an experience connected to religion or spirituality that a person experiences as harmful, threatening, coercive, betraying, or deeply disruptive. Spiritual or religious trauma: lasting adverse responses that a person connects to overwhelming or deeply disruptive religious or spiritual experiences. This is descriptive, not a diagnosis. | “the mistreatment of a person who is in need of help, support, or greater spiritual empowerment, with the result of weakening, undermining, or decreasing that person’s spiritual empowerment.”

**Core guidance:** Spiritual/religious abuse centers misuse of power or authority, coercion, control, exploitation, intimidation, silencing, or punishment for dissent. The terms religious abuse and spiritual abuse often overlap. Use the participant’s preferred language. None of these terms alone establishes a diagnosis.

**Stephanie confetti:** Keep the broad umbrella language because much harm occurs in religious organizations while some occurs in families, relationships, recovery communities, or spiritual settings.

**Source anchors:** Ellis et al. (2022).

## Slide 9: Spiritual Trauma Can…

**Participant-facing content:** Spiritual Trauma Can…

**Core guidance:** Present these as possible responses. Ask “Does any of this fit?” rather than “Which symptoms do you have?” Do not solicit details.

**Stephanie confetti:** “Hijacked sense of safety” may remain as accessible informal language.

**Source anchors:** Ellis et al. (2022); SAMHSA TIP 57 (2014).

## Slide 10: A Quick Word About Spiritual Bypassing

**Participant-facing content:** A Quick Word About Spiritual Bypassing | Spiritual bypassing happens when spiritual beliefs, practices, or language are used to avoid, suppress, invalidate, or prematurely close painful emotional or practical work. Prayer, scripture, forgiveness, and meaning-making are not inherently bypassing. Function and context matter. Examples include platitudes or “Band-Aid Bible Verses” used to dismiss pain or avoid accountability.

**Core guidance:** Bypassing is a functional concept, not a diagnosis. The same practice can support one person and silence another depending on timing, power, consent, and function.

**Stephanie confetti:** Keep “Band-Aid Bible Verses” and “Everything happens for a reason” as recognizable examples. The problem is the dismissive use, not scripture itself.

**Source anchors:** Cashwell et al. (2007); Fox et al. (2017).

## Slide 11: What Can Spiritual Abuse Look Like?

**Participant-facing content:** What Can Spiritual Abuse Look Like? | Bullying, manipulation, humiliation, condemnation, excessive demands, surveillance, threats, or punishment for dissent Using spiritual authority, dependency, or fear to control another person Using beliefs or scripture to justify exploitation or abuse Betrayal by a trusted sacred person, relationship, or system can deepen the impact.

**Core guidance:** Separate abusive tactics from trauma outcomes. Betrayal by a trusted sacred system may deepen impact but is not universal.

**Stephanie confetti:** Use ordinary examples before extreme cases so participants can recognize patterns without sensationalizing.

**Source anchors:** Ellis et al. (2022).

## Slide 12: What Can Spiritual Abuse Look Like?

**Participant-facing content:** What Can Spiritual Abuse Look Like? | Claiming exclusive divine authority to control others Using beliefs to shame or humiliate Making acceptance conditional on performance Financial exploitation Physical or sexual abuse | Using fear to force compliance Using beliefs to justify abuse Isolation or threatened isolation Punishment for questions or dissent Belittling other beliefs Discrimination tied to gender, race, sexuality, or identity

**Core guidance:** These categories are examples, not a checklist that diagnoses a person or faith community. Note coercion, dependency, retaliation, surveillance, and blocked access to care.

**Stephanie confetti:** Invite participants to substitute examples from their own cultural context without making them disclose personal experience.

**Source anchors:** Ellis et al. (2022).

## Slide 13: Examples and Red Flags

**Participant-facing content:** Examples and Red Flags | Repeatedly blaming or shaming you when prayers are not answered Silencing reports of abuse or protecting people who caused harm Trying to stop you from practicing your religion or pressuring you to practice one Treating mental-health concerns only as weakness, sin, or spiritual warfare Rejecting or isolating someone to enforce compliance

**Core guidance:** If someone recognizes possible current abuse, do not investigate in group. Arrange private follow-up, assess immediate safety, and follow facility and mandated-reporting policy.

**Stephanie confetti:** The prayer example should focus on repeated blame, shame, or conditional acceptance rather than one insensitive remark.

**Source anchors:** SAMHSA (2014).

## Slide 14: Examples and Red Flags

**Participant-facing content:** Examples and Red Flags | Presenting unqualified “counseling” as professional care, obstructing needed treatment, or misusing confidentiality Mocking or degrading someone because of belief or nonbelief Forcing religious learning or practice Using religious texts to manipulate someone into accepting physical, financial, emotional, or sexual abuse

**Core guidance:** Unlicensed or pastoral counseling is not automatically abusive. Red flags include misrepresented competence, coercion, obstruction of needed care, misuse of confidentiality, or using the relationship to facilitate harm.

**Stephanie confetti:** Preserve the directness of the scripture-manipulation example.

**Source anchors:** ACA Code of Ethics; Ellis et al. (2022).

## Slide 15: Religious Trauma Syndrome: What Does the Term Mean?

**Participant-facing content:** Religious Trauma Syndrome: What Does the Term Mean? | Some people use “religious trauma” or “Religious Trauma Syndrome” to describe distress linked to harmful or authoritarian religion or to leaving it. RTS is a practitioner and community term, not a recognized diagnosis. It is not equivalent to “PTSD + complex PTSD.” A clinician can assess the person’s actual concerns and any recognized diagnoses that may apply.

**Core guidance:** RTS is not in DSM-5-TR or ICD-11 and has no accepted diagnostic criteria. Participants may encounter the term online or in practice communities. Assess actual symptoms, functioning, grief, depression, anxiety, PTSD when criteria are met, religious/spiritual struggle, betrayal, identity disruption, and meaning disruption.

**Stephanie confetti:** You can say: “The term may help somebody find a community, but it cannot do the clinician’s whole job.”

**Source anchors:** APA DSM-5-TR (2022); WHO ICD-11; Winell (2011) only as provenance.

## Slide 16: People may not recognize spiritual abuse while it is happening. Dependency, isolation, information control, betrayal, grooming, or punishment for questioning can create fear and confusion.

## Harm can also involve stripping away someone’s right to interpret and tell their own story. Reclaiming that right can be part of healing.

**Participant-facing content:** People may not recognize spiritual abuse while it is happening. Dependency, isolation, information control, betrayal, grooming, or punishment for questioning can create fear and confusion. Harm can also involve stripping away someone’s right to interpret and tell their own story. Reclaiming that right can be part of healing.

**Core guidance:** People differ in awareness and response. Explain how coercive environments can narrow information, options, and perceived safety. Preserve narrative agency.

**Stephanie confetti:** Avoid language suggesting that participants were gullible or passively brainwashed.

**Source anchors:** Ellis et al. (2022).

## Slide 17: Bigotry and Abuse Can Overlap

**Participant-facing content:** Bigotry and Abuse Can Overlap | Bigotry involves prejudice or hostility toward people because of group membership. It can be harmful and can contribute to traumatic responses. Spiritual or religious abuse involves harmful uses of power, authority, coercion, control, exploitation, dependency, intimidation, or punishment for dissent. Context, pattern, and impact matter. Bigotry may occur within an abusive relationship or system.

**Core guidance:** Teach overlap, not a binary. Bigotry can be an adverse or traumatic experience and can function as part of abuse. Abuse does not require an explicit threat of physical harm.

**Stephanie confetti:** The Naked Pastor cartoon may stay if copyright/use is appropriate. Keep the distinction memorable without minimizing bigotry.

**Source anchors:** Ellis et al. (2022).

## Slide 18: Visual / transition slide

**Participant-facing content:** Spiritual Trauma

**Core guidance:** Intentional session hinge. If splitting the curriculum, summarize Part 1, name support options, and offer responsive grounding if needed. On return, recap without asking for disclosures.

**Stephanie confetti:** Keep the exact repeated book collage. The repetition is structural, not accidental.

**Source anchors:** SAMHSA (2014).

## Slide 19: Spiritual Harm & Abuse Scale – Clinical Screener (SHAS-CS)

**Participant-facing content:** Spiritual Harm & Abuse Scale – Clinical Screener (SHAS-CS) | A Christian-context conversation aid that can help people name harmful experiences and internal responses. It is not a diagnostic instrument and does not determine whether trauma or abuse occurred. Its categories may help identify topics to explore privately with a qualified clinician.

**Core guidance:** The SHAS-CS is a Christian-context conversation aid. It is not diagnostic and does not determine whether abuse or trauma occurred. Its public score bands are sample-based, not clinical cutoffs. Prefer voluntary, private, item-level discussion.

**Stephanie confetti:** Use it for recognition and vocabulary in this Southern U.S. context without turning the group into a scoring exercise.

**Source anchors:** Koch & Edstrom (2022); SHAS-CS v1.3.

## Slide 20: Visual / transition slide

**Core guidance:** If showing the instrument, do not ask for a show of hands or public scores. Explain that participants may skip items and that non-Christian experiences may not fit the tool.

**Stephanie confetti:** The visual can prompt “Which categories exist?” without asking “What happened to you?”

**Source anchors:** Koch & Edstrom (2022).

## Slide 21: Categories Represented in the SHAS-CS

**Participant-facing content:** Categories Represented in the SHAS-CS | Controlling Leadership: elevated leaders claim unusual access to God and extensive influence over daily decisions. Embracing Violence: fear, terror, violent imagery, or scripture may be used to motivate compliance or justify harm. Maintaining the System: victim-blaming, shunning, protecting leaders, silencing concerns, and social isolation may preserve the status quo.

**Core guidance:** Call these scale domains, not types of trauma. They mix system characteristics, experiences, and responses.

**Stephanie confetti:** Use local or familiar examples carefully; avoid naming living organizations or adjudicating theology.

**Source anchors:** Koch & Edstrom (2022).

## Slide 22: Categories Represented in the SHAS-CS

**Participant-facing content:** Categories Represented in the SHAS-CS | Gender Discrimination: opportunities, authority, or worth may be restricted because of gender. Internal Distress: negative religious experiences may be connected with grief, anger, anxiety, shame, isolation, self-image concerns, or loss of meaning. Harmful God-Image: a person may experience fear, anger, betrayal, or distrust in relation to God or the divine.

**Core guidance:** The original factor structure did not capture every identity-based harm. Do not universalize the gender factor or assume a particular God-image. Use participant language.

**Stephanie confetti:** “God has become the villain” may remain as accessible Stephanie confetti after the neutral wording, if it fits the room.

**Source anchors:** Koch & Edstrom (2022).

## Slide 23: Possible Effects and Struggles

**Participant-facing content:** Possible Effects and Struggles | Anger toward God, a faith community, or authority Existential distress or loss of meaning Difficulty connecting with other people Feeling forgotten, betrayed, or abandoned Difficulty trusting others or your own judgment Shame or reduced self-worth Questions about identity, values, beliefs, or purpose These responses are possible, not inevitable.

**Core guidance:** Use may/can language. Include grief, betrayal, shame, anger, isolation, and meaning disruption while leaving room for resilience and mixed feelings. Fight/Flight/Freeze/Fawn and its examples are an accessible, popular explanatory framework—not a formal diagnostic taxonomy. Do not imply that a listed behavior maps uniquely or inevitably to one response.

**Stephanie confetti:** Ask participants to notice, not disclose: “Which words are familiar, if any?”

**Source anchors:** Ellis et al. (2022); Exline et al. (2014).

## Slide 24: Great…. Now what?

**Participant-facing content:** Great…. Now what?

**Core guidance:** This transition restores choice after naming harm. State clearly that no spiritual conclusion is required.

**Stephanie confetti:** Build the "You can have Jesus and ___, too" list with medication, therapy, grief, emotions, questions, science, boundaries, the 12 Steps, or secular recovery. Keep it responsive to the group.

**Source anchors:** SAMHSA (2014).

## Slide 25: Healing from Spiritual or Religious Harm

**Participant-facing content:** Healing from Spiritual or Religious Harm | Name what happened in language that fits you. Your experience can matter without accepting a trauma label. Acknowledge your reactions and needs. Validation does not require someone else to agree with every interpretation. Consider individual support or therapy if it would help. Treatment choices should reflect your needs, preferences, and clinical assessment.

**Core guidance:** Participants do not have to accept a trauma label. Therapy is one option, not a requirement. Use assessment, preference, readiness, culture, access, and clinical fit when discussing care.

**Stephanie confetti:** Medication/ED anecdote: use humor about inconsistent attitudes toward psychiatric versus physical-health treatment, then land the serious point that symptom treatment can restore enough capacity for choice and recovery. Another facilitator may substitute an authentic example.

**Source anchors:** VA/DoD (2023); SAMHSA (2014).

## Slide 26: Healing from Spiritual or Religious Harm

**Participant-facing content:** Healing from Spiritual or Religious Harm | Let yourself feel complicated emotions, even when other people do not understand them. Grieve what you lost. Questions, doubt, and confusion are allowed. Healing is not linear. The abuse was not your fault. You still retain choice about the pace, direction, and support that fit your healing.

**Core guidance:** Preserve fault versus responsibility/agency: abuse was not their fault, and they retain choice over pace, support, and next steps. Do not imply they caused their need to heal.

**Stephanie confetti:** "Your whole system may be recalibrating" can remain as metaphor, not biological fact.

**Source anchors:** SAMHSA (2014).

## Slide 27: Healing from Spiritual/Religious Trauma

**Participant-facing content:** Healing from Spiritual/Religious Trauma | Reclaim Your Values and Beliefs Rebuilding Trust: in others, your Higher Power, and yourself Creating Supportive Networks Exploring New Perspectives Examining religious trauma does not necessarily mean rejecting or opposing all religion or spirituality Don't shame yourself if you do choose to walk away

**Core guidance:** Faith-preserving, faith-redefining, uncertain, plural, and nonreligious outcomes are all permissible. Trust, forgiveness, and reconciliation are optional and may be gradual or inappropriate.

**Stephanie confetti:** Your Higher Power is not subject to someone else's interpretation. Keep the distinction between personal belief and institutional worship.

**Source anchors:** Pargament (2007); SAMHSA (2014).

## Slide 28: Healing from Spiritual/Religious Trauma

**Participant-facing content:** Healing from Spiritual/Religious Trauma | Rewrite Your Story Articulating a New Narrative: Re-authoring your life story to include your experiences, struggles, and triumphs Each person's path to healing will be unique, and it's important to find what works best for you.

**Core guidance:** Narrative re-authoring is an invitation, not a demand for a positive ending. Allow ambiguity, grief, anger, and unfinished stories. Avoid detailed group processing.

**Stephanie confetti:** Use the creator's ordinary, human language. This slide can be brief because the conversation does the work.

**Source anchors:** White & Epston (1990); SAMHSA (2014).

## Slide 29: Visual / transition slide

**Core guidance:** These three workbooks are creator-used resources: Stephanie owns them and has personally worked from them. They are not an exhaustive, ranked, or comprehensive resource list. Another facilitator should become familiar with each resource before teaching from it and may substitute a comparable resource they have personally reviewed and can responsibly discuss.

**Stephanie confetti:** Keep the three specific workbooks shown on the slide.

**Source anchors:** See verified bibliography.

## Slide 30: Established and Population-Specific Trauma Treatments

**Participant-facing content:** Established and Population-Specific Trauma Treatments | ESTABLISHED ADULT PTSD PSYCHOTHERAPIES 1. Cognitive Processing Therapy (CPT) 2. Prolonged Exposure (PE) 3. Eye Movement Desensitization and Reprocessing (EMDR) Highlighting indicates local availability, not superiority or fit for everyone. | POPULATION-SPECIFIC OR DISTINCT PROTOCOLS 4. Trauma-Focused CBT (TF-CBT): children/adolescents and caregivers 5. Narrative Exposure Therapy (NET): distinct from Narrative Therapy 6. DBT-PTSD / DBT-PE: distinct from standard DBT

**Core guidance:** CPT, PE, and EMDR have strong guideline support for adult PTSD. TF-CBT is designed for children/adolescents and caregivers. NET is distinct from generic Narrative Therapy. DBT-PTSD and DBT-PE are specialized protocols. Highlighting means local availability only.

**Stephanie confetti:** Do not quote floating success percentages. Ask: “What kind of help is this, who is it for, and is it available here?”

**Source anchors:** VA/DoD (2023); NICE NG116; Cohen et al. (2017).

## Slide 31: Other Approaches Participants May Encounter

**Participant-facing content:** Other Approaches Participants May Encounter | BROADER OR EMERGING THERAPIES 1. Somatic Experiencing 2. Internal Family Systems 3. Narrative Therapy 4. AEDP ADJUNCTIVE APPROACHES 5. Trauma-informed yoga 6. Expressive arts (not currently offered locally) 7. Equine-Assisted Services — AVAILABLE HERE: weekly group and individual sessions with the equine facilitator | CONCEPTUAL FRAMEWORK 8. Polyvagal Theory: debated; not a psychotherapy or settled biological explanation INVESTIGATIONAL 9. Psychedelic-assisted approaches: lawful, professionally supervised research or regulated care only; never independent substance use

**Core guidance:** Broader therapies and adjuncts may help trauma-related difficulties without having evidence equivalent to guideline-supported first-line PTSD psychotherapies. Equine-Assisted Services are currently available here through a weekly group and individual sessions with the equine facilitator; present them as adjunctive, not first-line PTSD psychotherapy. The facility no longer offers an art-specific group or individual art session. Expressive arts may remain as an educational example, but must not be highlighted as locally available. Polyvagal Theory is a debated framework, not psychotherapy. Psychedelic-assisted care remains investigational for PTSD in the U.S.; distinguish lawful supervised research/regulated care from independent substance use.

**Stephanie confetti:** Clearly separate evidence category from local availability. Availability does not equal superiority or fit, especially in an SUD setting.

**Source anchors:** FDA (2023, 2024); Grossman (2023); VA/DoD (2023).

## Slide 32: Six Principles of Trauma-Informed Care

**Participant-facing content:** Six Principles of Trauma-Informed Care | Safety Trustworthiness and Transparency Peer Support Collaboration and Mutuality Empowerment, Voice, and Choice Cultural, Historical, and Gender Issues

**Core guidance:** These are SAMHSA’s six principles. They guide how care is delivered; they are not a psychotherapy or proof of effectiveness for a particular treatment. Apply them to this group through choice, transparency, collaboration, and cultural humility.

**Stephanie confetti:** Therapist-has-a-therapist comment is optional confetti that humanizes clinicians; it is not an empirical rule.

**Source anchors:** SAMHSA (2014).

## Slide 33: Visual / transition slide

**Participant-facing content:** Take a Nap | Stephanie Kepley

**Core guidance:** Before the joke, orient participants to follow-up support and remind them that recovery takes time. Do not imply that rest alone treats trauma.

**Stephanie confetti:** Preserve “Take a Nap” and “just not during programming.” It is the intentional closing beat.

**Source anchors:** No empirical citation required; intentional teaching device.

# Clinical response quick guides

| Situation | In the room | After group |
|---|---|---|
| Activation | Slow the pace; orient to choice; offer step-out or support; use a brief grounding option only if welcome. | Private check-in and connection to current treatment/support plan. |
| Detailed trauma disclosure | Thank the person, stop graphic detail gently, and explain the group boundary. | Private assessment and appropriate individual follow-up. |
| Possible current abuse or danger | Do not investigate publicly. Clarify that safety concerns require private follow-up. | Assess immediate risk; follow mandated-reporting and facility procedures. |
| Theological conflict | Protect dignity and stop cross-examination or recruitment. Focus on function, power, choice, and impact. | Consult only if conflict affected safety or treatment engagement. |
| SHAS item endorsement | Do not interpret publicly or solicit scores. | Explore privately if the participant chooses; review critical items for present safety. |
| Psychedelic question | Distinguish investigational, supervised, lawful treatment research from independent substance use. | Refer to qualified medical/clinical staff and current policy; never facilitate illicit acquisition. |

## Facilitator preparation checklist

- Review the deck notes and local crisis, reporting, referral, and documentation policies.
- Know which highlighted services are actually available at the facility and the referral pathway for each.
- Identify the Part 1 stopping point before group and protect time for closure.
- Choose only humor and examples that feel authentic. Omit Stephanie confetti that you cannot deliver naturally.
- Verify that current images/resources remain appropriate and permitted for educational use.